Patient Group Direction: **Sodium Chloride 0.9% Fluid Bolus**			
Version: **4.1**	Date of introduction: **18/02/2026**	▼Black Triangle Product: **NO**	Classification: **POM**

For the administration only of the above medicine(s) by Registered Practitioners employed by the Trust.

This patient group direction must be agreed to and signed by registered health care professionals involved in its use. The service lead or manager is responsible for holding the original signed copy in the clinical setting where the PGD will be used. An electronic copy can be found in the approved PGD database on the Trust intranet.

Developed / Reviewed by:		
Lead Author	**Name**: Joe Maguire **Signature:**	**Position**: Deputy Chief Pharmacist (MSO) **Date**: 16/02/2026
Physician/Consultant*	**Name**: Peter Riou **Signature**:	**Position**: Emergency Medicine - Consultant **Date**: 10/02/2026
Lead Nurse / Allied Health Professional/ Lead Practitioner	**Name**: Rachel Toft / Fiona Heckles **Signature**:	**Position**: DART Lead (N) **Date**: 16/02/2026
Pharmacist*	**Name**: Emma Cunnane **Signature**:	**Position**: Lead Pharmacist for Acute Medicine **Date**: 17/02/2026
ALL signature boxes above MUST be complete before submission to Medicine Management Group for approval.		
Organisational authorisation by MMG (Chair, or Deputy)	**Name**: Joe Maguire **Signature**:	**Position**: Chief Pharmacist **Date**: **18/02/2026** **PGD Expiry Date: Feb 2029** (no longer than 3 years)

*For PGDs covering systemic antibiotic use, a microbiologist and/or antimicrobial pharmacist must be included in its development.

Version	Summary of Changes
4.1 (Replaces 4.0)	• Addition of new indication - anaphylaxis (resus). • Additional exclusion of angioedema in anaphylaxis. • Total accumulative dose moved to quantity to administer field with new maximum for anaphylaxis. • References updated to include Resus Council Anaphylactic (2021) & Resusitation (2025) Guidelines. • Anaphylaxis changes made in consultation with Ben Clayton, Resus Lead Consultant.

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You must be authorised by name under the current version of this PGD before you attempt to work according to it.

1. Clinical Condition

Indication *add "[off-label]" if it is an indication outside of the manufacture's license*	For treatment of adults with: • Hypovolaemia in deteriorating adult patients who present with signs of shock (including anaphylaxis, DKA & sepsis) • Cardiorespiratory arrest in acute emergency situations *[off-label]*
Inclusion criteria	Hypovolaemic adults including those with: • Systolic BP < 100mmHg • Pulse > 90bpm • Capillary refill > 2 secs or cool peripheries • Respiratory rate >20 breaths per minute • NEWS Score ≥ 5
Exclusion criteria	Contraindications: • Patients presenting with a history or signs of acute heart failure, or pulmonary oedema. • Patients with peripheral oedema or documented hypernatraemia or chronic hyponatraemia, not including angioedema in the setting of anaphylaxis • Hypersensitivity to any ingredient in Sodium Chloride 0.9% intravenous infusion or packaging.
Cautions / Need for further advice	Verify the treatment plan with medical staff before starting the infusion in the following cases, unless the situation is immediately life-threatening: • Impaired renal function • Patients with hepatic failure • Hypertension and pre-eclampsia May cause fluid over load and oedema.
Action if excluded	Refer to medical practitioner (or non-medical prescriber if appropriate) or alternative action as indicated by related protocol. Document reason for exclusion in patient's records.
Action if patient declines treatment/refuses medication	• Document informed refusal in patient's records.

2. Description of Treatment

Method or route of administration	Intravenous infusion
Dose	• Administer a rapid fluid challenge of 500mLs over <15 minutes. • A further 250-500mLs of 0.9% sodium chloride solution may be considered if the patient does not improve after 20 minutes. • Ensure that urgent review and assessment by medical practitioner takes place.
Frequency	Single dose, may be repeated (see quantity to administer below).
Duration of treatment	<50 minutes
Quantity to supply / administer	Administration only under this PGD (no supply). • Up to a maximum of 1000mLs (continuous monitoring of patient required) • In anaphylaxis, doses of 3000-5000mLs may be needed (by Resuscitation Practitioners only)
Storage	Store below 25°C. Protect from excessive heat and freezing.

3. Patient Information

Advice and information to patient/carer, including side effects	• Unusual or life-threatening reactions require immediate medical attention. • Explain the current course of action to patient and/or carer and the need for urgent medical assessment. • Highlight any known medical history to emergency services on hand over.
Follow-up advice to be given to patient or carer	Individual advice to be discussed on discharge from emergency department.

4. Staff Characteristics

Registered Profession (tick those that apply) *Nursing Associates not eligible to use PGDs	☐ Chiropodists & Podiatrists ☐ Dental hygienists ☐ Dental therapists ☐ Dieticians ☐ Midwives ☑ Nurses* ☐ Occupational Therapists ☐ Optometrists ☐ Orthoptists ☐ Orthotists & Prosthetists ☑ Paramedics ☐ Pharmacists ☐ Physiotherapists ☐ Radiographers ☐ Speech & Language Therapists
Additional Qualifications	Nil in addition to above registration.
Specialist Competencies	Immediate Life Support or Advanced Life support (ILS/ALS) in date qualification/instructor status. OR Register Practitioners who have successfully passed within the last three years, the HEAC332 Care of Critically Ill Adults in Non-critical care Acute Area
Additional Requirements	• Evidence of CPD, and any training/competences relevant to the PGD. • Working knowledge of organisational policies (including Medicines Management Policy), Anaphylaxis Guidance (resus.org.uk) & Safeguarding Policies and related mandatory training. • Evidence of continuing professional development, (and any training and competence relevant to this PGD).

5. Record keeping, Referral Arrangements and Audit Trail

Specify method of recording supply /administration including audit trail	• Document allergies and other adverse drug reactions clearly in patient records and inform the GP and other relevant practitioners/patient/carer for further reporting and action if required. • Report any adverse drug reactions to the (MHRA) through the yellow card reporting system (www.mhra.gov.uk). The following will be recorded in the patient's health records: • The dose and volume administered.

References used in the development of this PGD:	
	• BNF (British National Formulary) online accessed – 04 / 01 / 2023 • Summary of Product Characteristics (v.03 May 2022 Baxter Healthcare) accessed 04/01/2022 • NICE (2017) Intravenous Fluid Therapy in Adults In Hospital' clinical guideline CG74. https://www.nice.org.uk/guidance/cg174 • NICE (2016) - Algorithms for IV Fluid Therapy in Adults. https://www.nice.org.uk/guidance/cg174/resources/intravenous-fluid-therapy-in-adults-in-hospital-algorithm-poster-set-191627821 • Royal College of Physicians (2017) – National Early Warning Score (NEWS) Guidance. https://www.rcplondon.ac.uk/projects/outputs/national-early-warning-score-news-2 • Resuscitation Council (2025) - Adult In-Hospital Resuscitation Guideline. https://www.resus.org.uk/sites/default/files/2025-10/Adult%20in%20hospital%20algorithm%202025.pdf • Resuscitation Council (2021) - Emergency Treatment of Anaphylaxis Guideline. https://www.resus.org.uk/sites/default/files/2021-05/Emergency%20Treatment%20of%20Anaphylaxis%20May%202021_0.pdf

Contacts for more information on the status of this document	
Royal Devon PGDs Pharmacy Administrative Office, Pharmacy Department RD&E Hospital (Wonford) Tel: 01392 402441	
PGD Template Version: ***2025-1***	*Template approved by MMG (March 2025)*

PGD Record Keeping
Expired PGDs including staff signed authorisation sheet(s) should be scanned and retained on a shared dept/service Trust network drive for a period after the expiry date as follows: - 25 years if it relates to children - 8 years if it relates to adults only - 10 years if relates to use of an implant

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PGD's DO NOT REMOVE INHERENT PROFESSIONAL OBLIGATIONS OR ACCOUNTABILITY

It is the responsibility of each professional to practice only within the bounds of their own competence and in accordance with their own Code of Professional Conduct.

Note to authorising Managers: A paper copy of the full PGD must be kept in the relevant clinical area together with this sign-off sheet. Authorised staff can be signposted to the intranet where they can view the document.

CLINICAL AREA	LOCATION / DEPARTMENT

I have read and understood the Patient Group Direction and agree to supply / administer this medicine only in accordance with this PGD.

Name of Registered Professional	Signature	Authorising Manager	Date
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